

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Atovaquone/Proguanil (Malarone), Doxycycline, and Mefloquine for the treatment of Malaria Prophylaxis

Summary of NICE / NICE CKS Guidelines for Malaria Prophylaxis

Overview of Malaria

- **Definition:** Malaria is a parasitic disease transmitted by Anopheles mosquitoes. The condition is caused by Plasmodium species parasites, with P. falciparum being the most dangerous.
- **Plasmodium species:** P. falciparum, P. vivax, P. ovale, P. malariae, and P. knowlesi are the main human pathogens.
- **Endemic regions:** Sub-Saharan Africa, parts of Asia, South America, and the Pacific region. Risk varies by destination, season, and altitude.

Assessment and Risk Stratification

- **Destination risk zones:** Refer to PHE (Public Health England) ACMP (Advisory Committee on Malaria Prevention) guidelines or WHO malaria map for current destination-specific recommendations.
- **Duration of travel:** Determine length of stay and timing of travel. Prophylaxis should start before exposure and continue after return.
- **Medical history:** Assess for contraindications including renal disease, hepatic disease, psychiatric disorders, cardiac conduction abnormalities, and G6PD deficiency (relevant for some agents).
- **Previous antimalarial use:** Enquire about previous tolerance and efficacy of antimalarial agents; consider rotating agents if previous tolerability issues.
- **Pregnancy and breastfeeding status:** Doxycycline is contraindicated in pregnancy and children under 12 years. Mefloquine carries teratogenic risk. Atovaquone/proguanil is Category C but evidence is limited.

Management and Selection of Prophylaxis

- **Destination-based choice:** PHE ACMP recommends regimens based on regional chloroquine resistance patterns. Chloroquine-resistant P. vivax regions may require different agents.
- **First-line options:** Atovaquone/proguanil or doxycycline for areas with chloroquine-resistant P. falciparum.
- **Second-line options:** Mefloquine for areas with chloroquine-resistant P. falciparum where first-line agents are contraindicated or not tolerated.
- **Dosing adherence:** Emphasize the importance of taking prophylaxis exactly as prescribed, including continuation after return from endemic areas.

Red Flag Symptoms and When to Seek Urgent Medical Advice

- **Fever in returning travelers:** Any traveler returning from an endemic area with fever should be investigated urgently for malaria. This is a medical emergency requiring immediate referral to hospital and blood film examination.

- **Pregnancy with exposure:** Pregnant women exposed to malaria require specialist assessment. Risk of malaria exceeds some medication risks, but specialist advice is essential.
- **Severe symptoms:** Seek urgent medical attention for severe headache, confusion, seizures, breathing difficulties, severe anemia, or signs of organ failure.

Patient Group Direction

for the supply/administration of

Atovaquone/Proguanil (Malarone)

for the treatment of

Malaria Prophylaxis

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Prophylaxis and treatment of malaria caused by Plasmodium falciparum in adults and children weighing 11 kg or more.
Inclusion criteria	Adults and children weighing ≥ 11 kg travelling to malaria-endemic areas Those requiring chloroquine-resistant P. falciparum prophylaxis Pregnant women (limited evidence but category C in established pregnancy)

	Patients with renal or hepatic impairment (mild to moderate) Those unable to tolerate doxycycline or mefloquine
Exclusion criteria	Children weighing <11 kg Hypersensitivity to atovaquone, proguanil, or any excipients Severe renal impairment (eGFR <30 mL/min) Severe hepatic impairment Concurrent use with certain antiretrovirals (may be contraindicated dependent on specific agent)
Cautions	Monitor for gastrointestinal symptoms; take with food or milky drink Patients with diarrhoea may have reduced absorption; consider alternative or seek specialist advice Use with caution in patients with renal impairment (dose adjustment may be needed for severe impairment) Use with caution in patients with hepatic impairment May interact with methotrexate, warfarin, and some antiretrovirals; counsel on concurrent medications Photosensitivity not typically associated with this agent
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Atovaquone 250 mg and Proguanil 100 mg tablets
Legal category	POM (Prescription Only Medicine)
Storage	Store below 25°C. Protect from light and moisture. Keep in original packaging.
Route/method of administration	Oral
Dose and frequency	Adults and children ≥11 kg: 1 tablet once daily Start dosing 1-2 days before entering endemic area Continue throughout stay in endemic area Continue for 7 days after leaving endemic area
Quantity to be administered and/or supplied	Number of tablets sufficient for duration of travel (typically 14-84 tablets depending on trip length)
Maximum or minimum treatment period	From 1-2 days before travel until 7 days after return from endemic area (maximum typically 28 weeks continuous use)

Adverse effects	Nausea and abdominal pain (common) Mouth ulcers or oral ulceration Headache Diarrhoea Dizziness Rash (occasionally) Hair loss (rare) Elevated liver enzymes (reversible)
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Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply patient information leaflet (PIL) provided with the antimalarial medication. Provide written advice on preventing mosquito bites and recognizing symptoms of malaria.
Follow-up advice to be given to patient or carer	Take the antimalarial medication exactly as prescribed, including continuation after returning home. Missing doses reduces protection significantly. Malaria can develop weeks or even months after return from an endemic area; seek immediate medical attention if fever, chills, or flu-like symptoms develop. Use effective insect bite avoidance: sleep under insecticide-treated

	bed nets, use insect repellents containing DEET (20-30%) or picaridin (20%), and wear protective clothing (long sleeves and trousers, especially at dusk and dawn). Avoid outdoor activities between dusk and dawn when Anopheles mosquitoes are most active. Doxycycline users: Avoid excessive sun exposure; use high SPF sunscreen (≥ 30) daily and wear protective clothing to prevent photosensitivity reactions. Doxycycline users: Take capsules/tablets with a full glass of water while sitting or standing; do not lie down for 30 minutes after taking to prevent oesophageal ulceration. Women taking doxycycline should be aware of reduced oral contraceptive efficacy; use additional contraception if necessary. Mefloquine users: Monitor for changes in mood or mental health (depression, anxiety, or unusual thoughts); report any concerning symptoms immediately to your doctor or pharmacist. Mefloquine users: Avoid driving or operating machinery if experiencing dizziness or neurological symptoms. Report any adverse effects (rash, severe abdominal pain, unusual bruising, hearing changes, or psychiatric symptoms) to your doctor or pharmacist immediately. If you become pregnant while taking or planning to take prophylaxis, contact your doctor immediately for specialist advice. Upon return, if fever develops within 1 year, inform your doctor immediately that you have travelled to a malaria-endemic area—malaria diagnosis should be considered urgently.
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Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance: Malaria: prevention in adults and children
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which			
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this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
1/11/25	31/10/26		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Patient Group Direction

for the supply/administration of

Doxycycline

for the treatment of

Malaria Prophylaxis

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Prophylaxis of malaria caused by Plasmodium falciparum in areas of chloroquine and multidrug resistance. Also indicated for treatment of uncomplicated P. falciparum and P. vivax malaria.
Inclusion criteria	Adults and children ≥ 12 years of age travelling to malaria-endemic areas with chloroquine-resistant P. falciparum Those requiring long-term prophylaxis (cost-effective for extended travel >1 month) Patients intolerant of atovaquone/proguanil Those with normal renal and hepatic function
Exclusion criteria	Children under 12 years of age (risk of permanent tooth discoloration and enamel hypoplasia)

	Pregnant or breastfeeding women (teratogenic; causes permanent discoloration of teeth in fetus) Hypersensitivity to tetracyclines Severe hepatic impairment Myasthenia gravis
Cautions	Photosensitivity: Advise strict sun protection (sunscreen SPF ≥ 30, protective clothing, avoidance of midday sun) as doxycycline increases photosensitivity risk Oesophageal ulceration: Take with full glass of water, remain upright for 30 minutes. Avoid taking before bed. Vaginal candidiasis: Advise women of increased risk; provide advice on prevention and when to seek treatment Interaction with oral contraceptives: May reduce effectiveness; advise use of additional contraception Antacids, iron supplements, and dairy products may reduce absorption; separate dosing by at least 2-4 hours Increased intracranial pressure: Rare but possible; counsel patients to report severe headache May worsen acne initially or improve it; reassure patient
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Doxycycline 100 mg capsules (or tablets)
Legal category	POM (Prescription Only Medicine)
Storage	Store at room temperature (15-25°C) in a dry place. Protect from light.
Route/method of administration	Oral
Dose and frequency	Adults and children ≥ 12 years: 100 mg once daily Start dosing 1-2 days before entering endemic area Continue throughout stay in endemic area Continue for 4 weeks after leaving endemic area
Quantity to be administered and/or supplied	Sufficient capsules/tablets for duration of prophylaxis (typically 14-112 capsules depending on trip length)
Maximum or minimum	From 1-2 days before travel until 4 weeks after return from endemic

treatment period	area
Adverse effects	Photosensitivity reactions (common in sunny destinations; mainly erythema and burning sensation) Nausea, vomiting, and abdominal discomfort (particularly if not taken with adequate water) Oesophageal ulceration (if taken improperly or lying down) Vaginal candidiasis (vaginal itching, discharge) Headache and dizziness Rash and photosensitive dermatitis Discoloration of tongue Rarely: hepatotoxicity, autoimmune hepatitis, and lupus-like syndrome

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply patient information leaflet (PIL) provided with the antimalarial medication. Provide written advice on preventing mosquito bites and recognizing symptoms of malaria.
Follow-up advice to be given to patient or carer	Take the antimalarial medication exactly as prescribed, including continuation after returning home. Missing doses reduces protection

	significantly. Malaria can develop weeks or even months after return from an endemic area; seek immediate medical attention if fever, chills, or flu-like symptoms develop. Use effective insect bite avoidance: sleep under insecticide-treated bed nets, use insect repellents containing DEET (20-30%) or picaridin (20%), and wear protective clothing (long sleeves and trousers, especially at dusk and dawn). Avoid outdoor activities between dusk and dawn when Anopheles mosquitoes are most active. Doxycycline users: Avoid excessive sun exposure; use high SPF sunscreen (≥ 30) daily and wear protective clothing to prevent photosensitivity reactions. Doxycycline users: Take capsules/tablets with a full glass of water while sitting or standing; do not lie down for 30 minutes after taking to prevent oesophageal ulceration. Women taking doxycycline should be aware of reduced oral contraceptive efficacy; use additional contraception if necessary. Mefloquine users: Monitor for changes in mood or mental health (depression, anxiety, or unusual thoughts); report any concerning symptoms immediately to your doctor or pharmacist. Mefloquine users: Avoid driving or operating machinery if experiencing dizziness or neurological symptoms. Report any adverse effects (rash, severe abdominal pain, unusual bruising, hearing changes, or psychiatric symptoms) to your doctor or pharmacist immediately. If you become pregnant while taking or planning to take prophylaxis, contact your doctor immediately for specialist advice. Upon return, if fever develops within 1 year, inform your doctor immediately that you have travelled to a malaria-endemic area—malaria diagnosis should be considered urgently.
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Key references

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Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
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Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Patient Group Direction

for the supply/administration of

Mefloquine (Lariam)

for the treatment of

Malaria Prophylaxis

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Prophylaxis and treatment of malaria in areas of chloroquine-resistant <i>P. falciparum</i> . Second-line agent due to psychiatric and neurological adverse effects.
Inclusion criteria	Adults and children >5 kg (dosing varies by weight) where mefloquine is indicated by destination risk Those intolerant of or with contraindications to atovaquone/proguanil or doxycycline Pregnant women where other agents contraindicated and specialist advice obtained (teratogenic risk, Category C) Those with normal cardiac conduction and no psychiatric history
Exclusion criteria	Hypersensitivity to mefloquine, quinine, or quinidine

	Current or recent (within 3 months) mefloquine use Psychiatric disorders including depression, anxiety, or psychosis Seizure disorders or on anticonvulsant therapy Cardiac conduction disorders (including QT prolongation) or family history thereof Severe renal or hepatic impairment Children <5 kg (dosing not established)
Cautions	Psychiatric effects: Monitor for depression, anxiety, confusion, agitation, paranoia, and psychotic symptoms. Counsel patients to report mood changes immediately. Neurological effects: Dizziness, vertigo, ataxia, and peripheral neuropathy can occur; counsel on avoiding dangerous activities (driving, operating machinery). Seizure risk: Use with caution in those with family history of seizures; can lower seizure threshold. Cardiac effects: Monitor QT interval in those with risk factors; baseline ECG may be indicated in high-risk patients. Gastrointestinal upset: Take with food and plenty of water. Haematological effects: Rare reports of thrombocytopenia; advise patients to report unusual bruising or bleeding. Not suitable for repeated or frequent travel to endemic areas due to side effect burden. Avoid alcohol, which may exacerbate neuropsychiatric symptoms. Pregnancy: Avoid if possible; evidence of teratogenicity in animal models. Specialist advice essential.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Mefloquine 250 mg tablets
Legal category	POM (Prescription Only Medicine)
Storage	Store at room temperature (15-25°C). Protect from light and moisture.
Route/method of administration	Oral
Dose and frequency	Adults: 250 mg (1 tablet) once weekly Start dosing 2-3 weeks before entering endemic area (allows

	assessment of tolerability) Continue weekly throughout stay in endemic area Continue for 4 weeks after leaving endemic area
Quantity to be administered and/or supplied	Sufficient tablets for duration of prophylaxis (typically 2-12 tablets depending on trip length)
Maximum or minimum treatment period	From 2-3 weeks before travel until 4 weeks after return from endemic area
Adverse effects	Neuropsychiatric effects (common): depression, anxiety, insomnia, nightmares, mood changes, psychosis Dizziness and vertigo (very common) Ataxia and loss of balance Nausea and vomiting (especially if taken on empty stomach) Headache Abdominal pain and diarrhoea Tinnitus and hearing loss (rare) Cardiac arrhythmias and QT prolongation (rare but serious) Seizures (in susceptible individuals) Visual disturbances Hair loss Thrombocytopenia (rare) Stevens-Johnson syndrome and toxic epidermal necrolysis (very rare)

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: - that valid informed consent was given - name of individual, address, date of birth and GP - name of healthcare practitioner - name and brand of medication - date of supply dose, form and route quantity supplied - advice given, including advice given if excluded or declines treatment - details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows:

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Patient information

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Follow-up advice to be given to patient or carer	Take the antimalarial medication exactly as prescribed, including continuation after returning home. Missing doses reduces protection significantly. Malaria can develop weeks or even months after return from an endemic area; seek immediate medical attention if fever, chills, or flu-like symptoms develop. Use effective insect bite avoidance: sleep under insecticide-treated bed nets, use insect repellents containing DEET (20-30%) or picaridin (20%), and wear protective clothing (long sleeves and trousers, especially at dusk and dawn). Avoid outdoor activities between dusk and dawn when Anopheles mosquitoes are most active. Doxycycline users: Avoid excessive sun exposure; use high SPF sunscreen (≥ 30) daily and wear protective clothing to prevent photosensitivity reactions. Doxycycline users: Take capsules/tablets with a full glass of water while sitting or standing; do not lie down for 30 minutes after taking to prevent oesophageal ulceration. Women taking doxycycline should be aware of reduced oral contraceptive efficacy; use additional contraception if necessary. Mefloquine users: Monitor for changes in mood or mental health (depression, anxiety, or unusual thoughts); report any concerning symptoms immediately to your doctor or pharmacist. Mefloquine users: Avoid driving or operating machinery if experiencing dizziness or neurological symptoms. Report any adverse effects (rash, severe abdominal pain, unusual bruising, hearing changes, or psychiatric symptoms) to your doctor or pharmacist immediately. If you become pregnant while taking or planning to take prophylaxis, contact your doctor immediately for specialist advice. Upon return, if fever develops within 1 year, inform your doctor immediately that you have travelled to a malaria-endemic area—malaria diagnosis should be considered urgently.

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Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
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